ST. MARGARET MATERNAL-FETAL MEDICINE CENTER

ANTENATAL LONGITUDINAL SURVEILLANCE REPORT - CONFIDENTIAL

PATIENT: MAYA LIN MRN: MRN-PT-2026-001 DOB: 1995-04-12

GESTATIONAL AGE: 31 weeks 4 days VISIT DATE: 2026-09-15 PHYSICIAN: Dr. Elena Vance, MD

1. CLINICAL MEASUREMENTS & VITAL SIGNS

Blood Pressure: 142/92 mmHg (Right Arm, Sitting, Verified)

Heart Rate: 82 bpm | SpO2: 99% on room air

Weight: 72.4 kg | Fundal Height: 31 cm (appropriate for GA)

Fetal Heart Rate: 144 bpm (regular, baseline normal)

2. POINT-OF-CARE URINALYSIS

Urine Protein: 1+ (dipstick automated reader)

Protein-to-Creatinine Ratio (UPCR): 0.34 mg/mg [Ref: < 0.30]

Leukocyte Esterase: Negative | Glucose: Negative | Nitrites: Negative

3. COMPREHENSIVE OBSTETRIC LABORATORY PANEL

Platelet Count: 168 x10^3/uL [Ref: 150 - 450]

AST (SGOT): 32 U/L [Ref: 10 - 40]

ALT (SGPT): 28 U/L [Ref: 7 - 35]

Serum Creatinine: 0.78 mg/dL [Ref: 0.40 - 0.90]

Hemoglobin: 11.8 g/dL | Hematocrit: 35.2%

4. ATTENDING CLINICIAN ASSESSMENT & RECOMMENDATION

Patient presents at 31 weeks 4 days with newly elevated systolic and diastolic blood pressure (142/92 mmHg). Mild proteinuria noted on automated urinalysis with UPCR borderline elevated at 0.34 mg/mg. Platelet count and transaminases currently within acceptable ranges. Recommendation: Initiate ambulatory blood pressure monitoring twice daily, order follow-up preeclampsia labs in 7 days, and advise patient on warning signs (severe headache, scotoma, epigastric pain).

AMBULATORY BLOOD PRESSURE LOG - MAYA LIN (MRN: MRN-PT-2026-001)

Previous 3-Visit Longitudinal Hemodynamic Trajectory:

2026-07-20 (GA 24.0w): BP: 122/78 mmHg | Protein: Negative | Platelets: 210 x10^3/uL

2026-08-18 (GA 28.1w): BP: 132/84 mmHg | Protein: Trace | Platelets: 192 x10^3/uL

2026-09-15 (GA 31.4w): BP: 142/92 mmHg | Protein: 1+ | Platelets: 168 x10^3/uL

Physician Signature: Dr. Elena Vance, MD, FACOG (Electronic Stamp 2026-09-15 14:32 EST)